

Suicide Risk in BDD

Thoughts and attempts of suicide, occasionally successful, are frequent in individuals with Body Dysmorphic Disorder (BDD). Current or past suicidal ideation should thus be routinely assessed by the practitioner. Particular monitoring may be needed with younger patients, as antidepressant treatment can increase impulsivity and the likelihood of suicidal attempts.

Treatment

As noted earlier, many patients with BDD seek dermatologic, surgical, or other nonpsychiatric treatments during the course of their illness. Evidence indicates that such interventions do not lead to improvement; patients are usually disappointed with the results and may even experience a worsening of symptoms. Therefore, these forms of treatment should be avoided.

BDD is a severe and complex disorder that often requires multimodal treatment, including cognitive behavioral therapy (CBT), exposure and response prevention (ERP), and pharmacotherapy.

Pharmacologic trials demonstrate a good response to selective serotonin reuptake inhibitors (SSRIs), which reduce distress, compulsions, and the frequency and intensity of obsessions about perceived defects. SSRIs also improve insight. Notably, they appear equally effective in both delusional and nondelusional patients. Relatively high doses are typically required, with a therapeutic effect usually observed after 12 to 16 weeks of treatment. Close monitoring for side effects and adherence is essential, as poor compliance is a common cause of treatment failure.

A significant proportion of patients who do not respond to one adequate SSRI trial may benefit from a trial with another SSRI, suggesting that sequential trials of these agents should be considered in initially refractory cases. Other medications—such as antipsychotics (including pimozide), tricyclic antidepressants (except clomipramine), benzodiazepines, and anticonvulsants—have shown limited or no efficacy.

Prognosis and Clinical Course

BDD is a chronic condition with little evidence of spontaneous remission in the absence of treatment. While pharmacotherapy with appropriate agents can lead to improvement in many patients, it is important to emphasize that effective SSRI doses often exceed those approved by the U.S. Food and Drug Administration for other indications. Discontinuation of treatment is associated with high relapse rates, indicating that long-term maintenance therapy is frequently necessary. This should ideally include insight-oriented psychotherapy to support sustained recovery.

Self-Inflicted Dermatoses

Trichotillomania

Trichotillomania is currently classified as an impulse control disorder characterized by repetitive hair pulling, resulting in noticeable hair loss (alopecia). Diagnostic criteria include: - Increasing tension before pulling or when resisting the urge, - Pleasure, gratification, or relief when pulling out the hair, - Significant distress or impairment in social, occupational, or other areas of functioning.

Epidemiology

Estimated prevalence ranges from 0.5% to 3.5%, with a mean age of onset between 10 and 13 years. Despite its classification, trichotillomania shares notable clinical and neurobiological similarities with obsessive-compulsive disorder (OCD), which has implications for treatment. Common comorbid psychiatric conditions include depression, anxiety disorders, and OCD.

Clinical Findings

Trichotillomania presents with nonscarring alopecia, most commonly showing hairs broken at varying lengths. In some cases, repeated trauma may lead to scarring. Commonly affected areas include the scalp, eyelashes, eyebrows, and pubic region, with most patients pulling hair from multiple sites.

In rare cases, individuals may ingest the pulled hair, a behavior known as trichophagia, which can lead to the formation of a trichobezoar—a potentially life-threatening gastrointestinal complication.

The behavior is often performed automatically, with partial or full awareness, and typically occurs during sedentary, solitary activities. Patients frequently attempt to resist the urge and may go to great lengths to conceal the cosmetic effects.

Histopathologic Findings

Diagnostic histopathologic features in ambiguous cases may include: - Increased numbers of catagen-phase hairs, - Pigment casts around hair shafts, - Traumatized hair bulbs (often described as "frayed" or "matchstick" appearance), - Absence of significant inflammation or scarring.

These findings can help differentiate trichotillomania from other causes of alopecia.

Treatment